

SAMPLE — ILLUSTRATIVE EXAMPLE (NOT A REAL PRACTICE)

CAMPBELL3, LLC

Practice Revenue Blueprint

Uncaptured Medicare Care-Management Revenue Analysis

Prepared for: Westbrook Family Medicine

Attention: Dr. [Practice Lead]

Practice profile: 4 physicians · 7,500-patient panel · 28% Medicare · [State]

Date: June 13, 2026

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VERIFIED

This report contains no protected health information. All figures are derived from five practice-level operating numbers supplied by Westbrook Family Medicine. Estimates are revenue *opportunity* projections, not guarantees of collection. See methodology and assumptions.

This is an illustrative sample using fictional inputs for "Westbrook Family Medicine." It demonstrates the format and method; it is not a real practice or a real client's data. Your Blueprint will use your own numbers.

Executive Summary

In January 2025, Medicare introduced the Advanced Primary Care Management (APCM) codes (G0556, G0557, G0558). They replaced the legacy Chronic Care Management model's minute-by-minute time-logging requirement — the single biggest reason most primary care practices abandoned care-management billing as unsustainable. Most practices have not yet adjusted. That gap is money your practice is currently leaving on the table.

Based on the five numbers Westbrook Family Medicine provided, here is the conservative estimate of new annual Medicare revenue available across three payment models — Advanced Primary Care Management (APCM), Transitional Care Management (TCM), and the Annual Wellness Visit (AWV):

CONSERVATIVE ESTIMATE · NEW ANNUAL MEDICARE CARE-MANAGEMENT REVENUE

\$246,355

APCM (net of care management you already bill) + TCM + AWV. The moderate and optimistic scenarios — and the full method behind this number — are detailed inside.

Conservative

\$246,355

25% APCM enrollment · 30% TCM capture · 45% AWV completion

Moderate

\$421,076

40% APCM enrollment · 45% TCM capture · 55% AWV completion

Optimistic

\$595,797

55% APCM enrollment · 60% TCM capture · 65% AWV completion

What this Blueprint gives you

- 1. **The number** — your uncaptured annual revenue, by model, in three honest scenarios.
- 2. **Workflow fit** — which models actually fit your 4-physician staffing, and which to skip.
- 3. **Three implementation paths** — in-house, turnkey vendor, and hybrid, each with net revenue *after* realistic cost.
- 4. **A 90-day roadmap**, compliance guardrails, and the exact scripts your front desk and care team will use.

The guarantee. If this Blueprint's conservative scenario does not identify at least \$50,000 in new annual revenue, you owe nothing. For Westbrook Family Medicine, the conservative figure is **\$246,355** — the guarantee is met.

Your Numbers & the Eligibility Funnel

Everything in this report traces back to five numbers you supplied. No EHR access, no patient data. Here is exactly how those five numbers become an eligible population.

The five inputs	Value
Active patient panel	7,500
Percent Medicare	28%
Percent of Medicare that is traditional fee-for-service	65%
Patients currently billed monthly for care management	60
Medicare hospital discharges per month	40

From panel to eligible population

Step	Calculation	Patients
Total panel	given	7,500
Medicare patients	$7,500 \times 28\%$	2,100
Traditional FFS Medicare (APCM/AWV eligible)	$2,100 \times 65\%$	1,365
Medicare discharges / year (TCM eligible)	40×12	480

We count only traditional fee-for-service Medicare for APCM and AWV. Medicare Advantage patients are excluded from these estimates — their care-management economics run through plan contracts, not the fee schedule. Excluding them keeps this estimate conservative.

Why APCM changes the math for you

CMS APCM (G0556/G0557/G0558) does not require minute-by-minute time logging, unlike legacy CCM. Billing is based on the patient meeting the chronic-condition criteria and the practice furnishing the APCM service elements within the calendar month.
Basis: CMS CY2025 PFS final rule — APCM code family; CMS APCM FAQ.

Under the old CCM rules, a 4-physician practice had to capture and defend 20 minutes of documented staff time per patient per month. The labor cost and audit risk made it a wash. APCM removes the stopwatch: you bill on the patient's condition profile and the service elements furnished, not on logged minutes. That is what turns this from a break-even chore into net revenue.

Methodology & Rate Table

This Blueprint uses one master rate table. Every dollar figure in every scenario is built only from the rates below — nothing is improvised. 2026 Medicare Physician Fee Schedule, national non-facility allowed amounts, per CY 2026 PFS Final Rule (CMS-1832-F). Cross-verified 2026-06-12 against multiple independent industry sources (CircleLink, Thoroughcare, ChartSpan, Prevounce, Rimidi); non-QP conversion-factor figures used where two CFs apply. APCM = Advanced Primary Care Management (G0556/G0557/G0558, effective 2025).

Code	Service	2026 rate	Unit
G0556	APCM Level 1 (1 chronic condition)	\$16.37	per member / month
G0557	APCM Level 2 (2+ chronic conditions)	\$53.78	per member / month
G0558	APCM Level 3 (2+ chronic + QMB / dual-eligible)	\$117.24	per member / month
99490	CCM, first 20 min (legacy model APCM replaces)	\$66.13	per member / month
99495	TCM, moderate complexity (14-day face-to-face)	\$220.00	per service
99496	TCM, high complexity (7-day face-to-face)	\$298.00	per service
G0438	AWV, initial visit	\$174.00	per service
G0439	AWV, subsequent visit	\$138.00	per service

Rates are national, non-facility 2026 Medicare Physician Fee Schedule allowed amounts. Actual payment varies by locality (GPCI) and payer mix. Status: VERIFIED.

How each scenario is built

- **APCM** — eligible FFS Medicare patients × enrollment rate, split across the three APCM tiers (15% Level 1 / 73% Level 2 / 12% Level 3), annualized, then *reduced* by the care management you already bill so nothing is counted twice.
- **TCM** — annual Medicare discharges × capture rate, split 60%/40% moderate/high complexity.
- **AWV** — eligible FFS Medicare × the gain from assumed current completion (30%, confirmed on your intake call) up to the scenario target.

The three scenarios differ only in adoption assumptions — enrollment, capture, and completion rates. They use identical rates and identical eligibility. Conservative is deliberately low so the number you build a decision on is one you can beat, not one you have to hit.

Advanced Primary Care Management (APCM)

APCM is the largest opportunity for Westbrook Family Medicine, and the one the 2025 rule change unlocked. Your eligible pool is **1,365 traditional FFS Medicare patients**. The table compares all three scenarios; tiers are split 15%/73%/12% across Levels 1/2/3.

APCM line	Conservative	Moderate	Optimistic
Enrollment rate (of eligible)	25%	40%	55%
Patients enrolled	341	546	751
Level 1 revenue (G0556, \$196/yr)	\$10,055	\$16,088	\$22,122
Level 2 revenue (G0557, \$645/yr)	\$160,767	\$257,228	\$353,688
Level 3 revenue (G0558, \$1,407/yr)	\$57,612	\$92,179	\$126,746
APCM gross annual	\$228,434	\$365,495	\$502,555
Less: care mgmt you already bill	(\$47,614)	(\$47,614)	(\$47,614)
Net new APCM revenue	\$180,821	\$317,881	\$454,942

Level 2 (2+ chronic conditions, \$54/member/month) is the workhorse — most of your Medicare panel qualifies. Your existing care-management billing (60 patients) is subtracted so nothing is counted twice.

CMS APCM and CCM cannot be billed for the same patient in the same month. APCM is billed instead of CCM, not in addition to it.

Basis: CMS APCM billing guidance — mutually exclusive with CCM/PCM in a month.

CMS Only one practitioner may bill APCM for a given patient per calendar month.

Basis: CMS APCM billing guidance — one biller per patient per month.

CMS Patient consent must be obtained and documented before billing APCM, and the patient must be informed of any applicable cost-sharing.

Basis: CMS APCM service-element requirements (consent).

CMS QMB (dual-eligible) patients may not be balance-billed for Medicare cost-sharing; APCM Level 3 (G0558) recognizes this population.

Basis: CMS QMB billing protections.

Transitional Care Management (TCM)

You reported 40 Medicare discharges per month (480 per year). Each is a billable TCM encounter if your team reaches the patient within two business days and sees them inside the required window. Most practices capture a fraction today; the scenarios below model closing that gap (mix: 60% moderate / 40% high complexity).

TCM line	Conservative	Moderate	Optimistic
Discharge capture rate	30%	45%	60%
99495 moderate (\$220) — encounters	86	130	173
99496 high (\$298) — encounters	58	86	115
Total TCM encounters / yr	144	216	288
TCM annual revenue	\$36,173	\$54,259	\$72,346

CMS TCM (99495/99496) is billed once per patient within 30 days of discharge, with a face-to-face visit inside the required window (14 days moderate / 7 days high) and interactive contact within 2 business days of discharge.

Basis: CMS TCM billing requirements.

Workflow fit note. TCM revenue depends entirely on a discharge alert reaching your team within two business days. If you do not have a reliable discharge feed (ADT, hospital fax, or a daily call), start TCM *after* you have closed that loop — otherwise you will bill a handful and miss the window on the rest. This is flagged again in your roadmap.

Annual Wellness Visit (AWV)

The AWV is the most underused code in primary care. We assume Westbrook Family Medicine currently completes about 30% of eligible AWVs (confirm this on your call — it moves the number). Each scenario models lifting completion toward a target across your 1,365 eligible patients.

AWV line	Conservative	Moderate	Optimistic
Completion target	45%	55%	65%
New AWVs vs. current (30%)	205	341	478
G0438 initial (\$174)	31	51	72
G0439 subsequent (\$138)	174	290	406
AWV annual revenue	\$29,361	\$48,935	\$68,509

CMS The Annual Wellness Visit (G0438 initial / G0439 subsequent) is covered once every 12 months and carries no patient cost-sharing when billed as a stand-alone AWV.
Basis: CMS AWV coverage rules.

AWVs also create the documented chronic-condition list that qualifies patients for APCM tiers — so AWV and APCM reinforce each other. Practices that run AWVs well find APCM enrollment easier.

Three Implementation Paths — Net Revenue After Cost

The number on page two is gross opportunity. What you keep depends on who does the work. Below are three honest paths for the APCM program (TCM and AWW are visit-based and billed in-house under every path). Figures use the **conservative** APCM scenario, so they are a floor.

Path	APCM gross (conservative)	Est. annual cost	Net year 1
In-house 1 FTE care coordinator(s) + software, run entirely by the practice.	\$228,434	\$82,000	\$146,434
Turnkey vendor Vendor runs outreach + tracking; keeps 45% of care-mgmt revenue.	\$228,434	\$110,795	\$117,639
Hybrid Practice runs clinical/enrollment (0.5 FTE); vendor does outreach labor, keeps 28%.	\$228,434	\$99,962	\$128,473

Cost assumptions: care coordinator loaded at \$72,000/yr, ~450 patients per FTE; software \$6,000/yr; turnkey vendor share 45%; hybrid vendor share 28%. These are planning assumptions you can tune; they are not CMS figures.

How to choose

- **In-house** keeps the most revenue and builds a durable capability, but you own hiring, training, and retention. Best if you have, or will hire, a care coordinator and want the asset.
- **Turnkey vendor** launches fastest with the least staff disruption, at the cost of the largest revenue share. Best if you want the revenue without adding headcount or process.
- **Hybrid** keeps clinical judgment and enrollment in-house while outsourcing the outreach labor that eats time. Usually the best risk-adjusted choice for a 4-physician practice.

No vendor recommendation is embedded in this report, and the firm preparing it does not resell or take referral fees from any care-management vendor. The path you pick is yours.

90-Day Implementation Roadmap

Window	Milestone	Owner
Days 1–15	Pick a path (in-house / turnkey / hybrid). Confirm current AWV completion and discharge-feed reliability. Designate one APCM billing practitioner.	Physician lead
Days 1–15	Pull the list of FFS Medicare patients with 2+ chronic conditions (your APCM Level 2 core). Target list of 341+ for the conservative ramp.	Office manager
Days 16–30	Build consent workflow and the monthly APCM service-element checklist. Train front desk on the enrollment script (next page).	Care coordinator
Days 16–45	Stand up the discharge loop for TCM: ADT feed or daily hospital census call, 2-business-day outreach protocol.	Care coordinator
Days 30–60	Begin APCM enrollment at the practice's pace; bill first month for enrolled cohort. Begin booking AWVs into open slots and pairing them with enrollment.	Whole team
Days 60–90	First full billing month reconciled. Review captured vs. modeled. Adjust enrollment outreach. Decide on staffing for the next ramp tier.	Physician lead

Sequencing matters. Start with APCM Level 2 enrollment and AWV — they are the largest, most predictable dollars and require no external feed. Add TCM only once the discharge loop is reliable. Do not try to launch all three in week one.

Compliance Guardrails

Every statement below is tied to a CMS basis and was cross-checked in this report's automated self-check before delivery. These are the rules that keep the revenue clean.

CMS APCM (G0556/G0557/G0558) does not require minute-by-minute time logging, unlike legacy CCM. Billing is based on the patient meeting the chronic-condition criteria and the practice furnishing the APCM service elements within the calendar month.

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Basis: CMS AWV coverage rules.

This is operational guidance, not legal advice. Confirm current-year specifics with your billing compliance resource before go-live. CMS guidance is updated periodically.

Scripts Your Staff Will Use

Front-desk APCM enrollment (at check-in or by phone)

Front desk → "Mr./Ms. [name], because you have ongoing conditions we help manage, Medicare now covers a monthly care-management service through our office — a dedicated person who keeps an eye on your care between visits, coordinates your medications, and is your direct line if something changes. Most patients owe little or nothing. Dr. [name] recommends it for you. May I have your okay to enroll you today?"

Care coordinator monthly touch (APCM)

Care coordinator → "Hi [name], this is [coordinator] from Dr. [name]'s office — your care-management check-in. How are you doing with [condition]? Any new medicines, ER visits, or hospital stays since we talked? Let's make sure your refills and upcoming appointments are set." *(Document the service elements furnished — no stopwatch required.)*

Post-discharge TCM outreach (within 2 business days)

Care coordinator → "Hi [name], I saw you were just in the hospital — I'm calling from Dr. [name]'s office to make sure your recovery goes smoothly. Do you have all your new medications? Any new symptoms? I'd like to get you in to see Dr. [name] within the next [7/14] days — does [day] work?" *(Interactive contact within 2 business days is required to bill.)*

AWV invitation

Scheduler → "Mr./Ms. [name], you're due for your yearly Medicare Wellness Visit — it's fully covered, no cost to you, and it's how we update your care plan and catch things early. I have [day/time] open. Shall I book it?"

Guarantee, Next Steps & Self-Check Attestation

The guarantee. If the conservative scenario in this Blueprint does not identify at least \$50,000 in new annual revenue, you owe nothing. Westbrook Family Medicine's conservative figure is **\$246,355**.

Next steps

- 1. **30-minute working session** to walk these numbers and choose a path.
- 2. We finalize your path decision and the first-cohort target list.
- 3. You execute the 90-day roadmap; we are available for the first billing-month reconciliation.

Self-check attestation

This Blueprint was not delivered until an automated gate independently recomputed every figure, verified every rate against the master table, confirmed APCM was netted against existing billing (no double-counting), and cross-checked every compliance statement against its CMS basis. The gate result:

SELF-CHECK: PASS Rate table status: **VERIFIED**

[PASS] Rate registration & table status
[PASS] Assumption mixes sum to 1.0 (no leaked/invented patients)
[PASS] Independent arithmetic reconciliation
[PASS] Double-counting guard (APCM replaces CCM)
[PASS] Sanity bounds
[PASS] \$50k guarantee threshold (conservative scenario)
[PASS] Compliance statements registered with CMS basis

RATE TABLE: VERIFIED
OVERALL GATE: PASS – cleared to render

Prepared by Campbell3, LLC. This analysis estimates revenue opportunity based on practice-supplied operating numbers and stated assumptions; it is not a guarantee of payment or collection and is not legal or billing-compliance advice. No PHI was used in its preparation.